TELFAST 6 mg/mL
Fexofenadine hydrochloride
Oral Suspension

PEDIATRIC USE
FOR ORAL ADMINISTRATION

Pharmaceutical form and packaging
Oral Suspension.
Packages containing one 30 mL or one 60mL bottle with dosing syringe.
Not all pack size are marketed.

Product Description
White to beige uniform suspension with raspberry cream aroma.

Composition
Each mL contains:
fexofenadine hydrochloride (equivalent to 5.6mg of fexofenadine). 6mg excipients q.s to 1 mL
(propylene glycol, disodium edetate, xanthan gum, poloxamer 407, titanium dioxide, sodium phosphate monobasic monohydrate, sodium phosphate dibasic heptahydrate, potassium sorbate, artificial raspberry cream flavour, sucrose, xylitol and purified water).

PHARMACOLOGICAL CHARACTERISTICS
Pharmacodynamic properties
Wheal and Flare: Human histamine skin wheal and flare studies in adults following single and twice daily doses of 20 and 40 mg fexofenadine hydrochloride demonstrated that the drug exhibits an antihistamine effect by 1 hour, achieves maximum effect at 2 to 3 hours, and an effect is still seen at 12 hours. There was no evidence of tolerance to these effects after 28 days of dosing. The clinical significance of these observations is unknown.
Histamine skin wheal and flare studies in 7 to 12 year old subjects showed that following a single dose of 30 or 60 mg, antihistamine effect was observed at 1 hour and reached a maximum by 3 hours. Greater than 49% inhibition of wheal area, and 74% inhibition of flare area were maintained for 8 hours following the 30 and 60 mg dose.
No statistically significant increase in mean QTc interval compared to placebo was observed in 714 adult subjects with seasonal allergic rhinitis given fexofenadine hydrochloride capsules in doses of 60 to 240mg twice daily for 2 weeks. Pediatric subjects from 2 placebo controlled trials (n=855) treated with up to 60 mg fexofenadine hydrochloride twice daily demonstrated no significant treatment- or dose-related increases in QTc. In addition, no statistically significant increase in mean QTc interval compared to placebo was observed in 40 healthy adult subjects given fexofenadine hydrochloride as an oral solution at doses up to 400 mg twice daily for 6

days, or in 230 healthy adult subjects given fexofenadine hydrochloride 240 mg once daily for 1 year.
In subjects with chronic idiopathic urticaria, there were no clinically relevant differences for any ECG intervals, including QTc, between those treated with fexofenadine hydrochloride 180 mg once daily (n = 163) and those treated with placebo (n = 91) for 4 weeks.

Pharmacokinetic properties
The pharmacokinetics of fexofenadine hydrochloride in subjects with seasonal allergic rhinitis and subjects with chronic urticaria were similar to those in healthy subjects.

Absorption:
TELFAST tablets: Fexofenadine hydrochloride was absorbed following oral administration of a single dose of two 60 mg capsules to healthy male subjects with a mean time to maximum plasma concentration occurring at 2.6 hours post-dose. After administration of a single 60mg capsule to healthy adult subjects, the mean maximum plasma concentration (Cmax) was 131ng/mL. Following single dose oral administrations of either the 60 and 180 mg tablet to healthy adult male subjects, mean Cmax were 142 and 494 ng/mL, respectively. The tablet formulations are bioequivalent to the capsule when administered at equal doses. Fexofenadine hydrochloride pharmacokinetics are linear for oral doses up to a total daily dose of 240 mg (120 mg twice daily). The administration of the 60 mg capsule contents mixed with applesauce did not have a significant effect on the pharmacokinetics of fexofenadine in adults. Co-administration of 180 mg fexofenadine hydrochloride tablet with a high fat meal decreased the mean area under the curve (AUC) and (Cmax) of fexofenadine by 21 and 20% respectively.
Telfast oral suspension: A dose of 5 mL of Telfast oral suspension containing 30 mg of fexofenadine hydrochloride is bioequivalent to a 30 mg dose of Telfast tablets. Following oral administration of a 30 mg dose of Telfast oral suspension to healthy adult subjects, the mean Cmax was 118 ng/mL and occurred at approximately 1 hour. The administration of 30 mg Telfast oral suspension with a high fat meal decreased the AUC and the mean Cmax by approximately 30 and 47%, respectively in healthy adult subjects.

Distribution:
Fexofenadine hydrochloride is 60% to 70% bound to plasma proteins, primarily albumin and α1-acid glycoprotein.

Metabolism:
Approximately 5% of the total dose of fexofenadine hydrochloride was eliminated by hepatic metabolism.

Elimination:
The mean elimination half-life of fexofenadine was 14.4 hours following administration of 60 mg twice daily in healthy adult subjects.
Human mass balance studies documented a recovery of approximately 80% and 11% of the [14C] fexofenadine hydrochloride dose in the feces and urine, respectively. Because the absolute bioavailability of fexofenadine hydrochloride has not been established, it is unknown if the fecal component represents primarily unabsorbed drug or is the result of biliary excretion.

Special Populations:
Pharmacokinetics in renally and hepatically impaired subjects and geriatric subjects, obtained after a single dose of 80 mg fexofenadine hydrochloride, were compared to those from healthy subjects in a separate study of similar design.

Renally Impaired:
In subjects with mild to moderate (creatinine clearance 41-80 mL/min) and severe (creatinine clearance 11-40 mL/min) renal impairment, peak plasma concentrations of fexofenadine were 87% and 111% greater, respectively, and mean elimination half-lives were 59% and 72% longer, respectively, than observed in healthy subjects. Peak plasma concentrations in subjects on dialysis (creatinine clearance ≤10 mL/min) were 82% greater and half-life was 31% longer than observed in healthy subjects. Based on increases in bioavailability and half-life, a dose of 60 mg once daily is recommended as the starting dose in adult patients with decreased renal function. For pediatric patients with decreased renal function, the recommended starting dose of fexofenadine is 30 mg once daily for patients 2 to 11 years of age and 15 mg once daily for patients 6 months to less than 2 years of age [see Administration].

Hepatically Impaired:
The pharmacokinetics of fexofenadine hydrochloride in subjects with hepatic impairment did not differ substantially from that observed in healthy subjects.

Geriatric Subjects:
In older subjects (≥65 years old), peak plasma levels of fexofenadine were 99% greater than those observed in younger subjects (<65 years old). Mean fexofenadine elimination halflives were similar to those observed in younger subjects.

Effect of Gender:
Across several trials, no clinically significant gender-related differences were observed in the pharmacokinetics of fexofenadine hydrochloride.

Pre-clinical safety data
The carcinogenic potential of fexofenadine hydrochloride was assessed through studies with terfenadine and supporting pharmacokinetic studies which demonstrated the proper exposure to fexofenadine hydrochloride (through the area under the curve concentration plasma values – AUC). No evidence of carcinogenicity was observed in

rats and mice with terfenadine (up to 150 mg/kg/day), resulting in a plasma exposure to fexofenadine of up to 4 times the therapeutic value in humans (based on 60 mg of fexofenadine hydrochloride, twice a day).
Several *in vitro* and *in vivo* studies conducted with fexofenadine hydrochloride did not demonstrate mutagenicity. When fexofenadine hydrochloride was administered as 2000 mg/kg oral doses in acute toxicity studies conducted with several animal species, no clinical signs of toxicity and no effect on body weight or food intake were observed. No relevant treatment- related effects were observed in rodents after necropsy. Dogs tolerated 450 mg/kg, administered twice a day, for 6 months and did not exhibit any toxicity other than occasional vomit.

INDICATIONS

Seasonal Allergic Rhinitis

Telfast Oral Suspension is indicated for the relief of symptoms associated with seasonal allergic rhinitis in children 2 to 11 years of age.

Chronic Idiopathic Urticaria

Telfast Oral Suspension is indicated for treatment of uncomplicated skin manifestations of chronic idiopathic urticaria in children 6 months to 11 years of age.

CONTRAINDICATIONS

Telfast Oral Suspension is contraindicated for use in patients with hypersensitivity to any of its formula components.

HOW TO USE AND STORAGE PRECAUTIONS AFTER OPENING

Shake the bottle well before administering Telfast Oral Suspension.
It is recommended that the dosing syringe included in the carton be used.
After opening the package, Telfast Oral Suspension must be stored below 30°C in its original package.

ADMINISTRATION

Seasonal Allergic Rhinitis

Children 2 to 11 Years: The recommended dose of Telfast Oral Suspension is 30mg (5ml) twice daily. A dose of 30 mg (5 mL) once daily is recommended as the starting dose in pediatric patients with decreased renal function.

Chronic Idiopathic Urticaria

Children 6 months to less than 2 years: The recommended dose of Telfast Oral Suspension is 15 mg (2.5 mL) twice daily. For pediatric patients with decreased renal function, the recommended starting dose is 15 mg (2.5 mL) once daily.
Children 2 to 11 Years: The recommended dose of Telfast Oral Suspension is 30 mg (5 mL) twice daily. For pediatric patients with decreased renal function, the recommended starting dose is 30 mg (5 mL) once daily.

Necessary conduct when a dose is missed.

When a dose is missed, remember to use it as soon as possible; however, if it is near the time to take the next dose, skip the missed dose and resume your usual dosing schedule, always observing the interval indicated in administration. **Never double a dose.**

Equivalence between the product chemical compound and the active ingredient

Each mL of Telfast Oral Suspension contains 6 mg of fexofenadine hydrochloride, which is equivalent to 5.6 mg of fexofenadine base.

WARNINGS

Studies conducted with fexofenadine hydrochloride did not associate the use of the product with reduced attention when operating motor vehicles or machinery, alteration in sleep quality or other effects on the central nervous system.

Risk of using the medication via not recommended routes of administration.

No studies have been conducted on the effects of Telfast Oral Suspension when taken via not recommended routes of administration.

Therefore, for your own safety and for the best efficacy of this medication, the drug must only be taken orally.

Pregnancy

No studies of Telfast Oral Suspension have been conducted in pregnant women. Telfast Oral Suspension should not be used during pregnancy unless the potential benefits exceed the potential risks to the fetus.

In studies that assessed reproductive toxicity conducted in mice, fexofenadine did not affect fertility, was not teratogenic and did not affect pre or postnatal development.

Lactation

No studies of Telfast Oral Suspension have been conducted in breast feeding women. Telfast Oral Suspension must only be used by lactating women if the potential benefits exceed the potential risks to the child.

This medication must not be used by pregnant women without a physician or surgeon dentist's advice.

Risk category in pregnancy: category C.

USE IN CHILDREN AND OTHER RISK GROUPS

Children

The efficacy and safety of fexofenadine hydrochloride have not been established in children under 2 years of age for seasonal allergic rhinitis and in children under 6 months of age for chronic idiopathic urticaria.

Other risk groups

Adjustment of the dose for the elderly or for patients with hepatic failure is not required.

DRUG-DRUG INTERACTION

The concomitant administration of Telfast Oral Suspension with erythromycin or ketoconazole did not demonstrate any significant increase in the QTc interval. No difference in adverse effects was reported when these agents were administered combined or separately.

No fexofenadine-omeprazole interaction was observed. However, the administration of an antacid containing aluminum hydroxide and magnesium, approximately 15 minutes before receiving fexofenadine hydrochloride reduced bioavailability. It is recommended to allow an interval of approximately two hours between administrations of fexofenadine hydrochloride and antacids that contain aluminium hydroxide and magnesium.

ADVERSE REACTIONS

In placebo-controlled studies involving patients with seasonal allergic rhinitis and chronic idiopathic urticaria, adverse events were similar to those of patients treated with placebo or fexofenadine.

Most frequently reported adverse events by adults include: headache (>3%), drowsiness, dizziness and nausea (1 - 3%). The adverse events reported during the controlled studies involving patients with seasonal allergic rhinitis and chronic idiopathic urticaria, with an incidence lower than 1% and similar to those of the placebo group and that were rarely reported after commercialization include: fatigue, insomnia, nervousness, alteration in sleep quality or nightmares. Patients also reported rare cases of exanthema, urticaria, itching and hypersensitive reactions, such as: angioedema, thoracic rigidity, dyspnea, redness and systemic anaphylaxis.

The adverse events reported in placebo- controlled studies of chronic idiopathic urticaria were similar to those reported in placebo-controlled studies of allergic rhinitis.

In placebo-controlled studies in children from 6 to 11 years of age with seasonal allergic rhinitis, adverse events were similar to those observed in clinical studies involving adults and children 12 years of age or older with seasonal allergic rhinitis.

In clinical controlled studies involving pediatric patients from 6 months to 5 years of age, no unexpected adverse events were observed in patients treated with fexofenadine hydrochloride.

OVERDOSAGE

Symptoms

Most of the fexofenadine hydrochloride overdose reports presented restricted information. However, dizziness, drowsiness and dry mouth were reported. In adults a single dose of up to 800 mg and doses of up to 690 mg, twice a day, for one month or 240 mg daily, for 1 year, were studied in healthy volunteers without the emergence of any clinically significant adverse events compared to the placebo. The maximum tolerated dose of Telfast Oral Suspension has not been established.

There were no deaths of mice that received oral doses of fexofenadine hydrochloride of up to 5000 mg/kg (110 times the maximum daily oral dose for adults and children on a mg/ m2 base) and in rats with oral doses of up to 5000 mg/ kg (230 times the maximum daily oral dose for adults and 210 times the maximum daily oral dose for children on a mg/m2 base). Additionally, no clinical signals of toxicity were observed; nor were there any significant pathologic findings. In dogs, no evidence of toxicity was observed with oral doses of up to 2000 mg/kg (300 times the maximum daily oral dose for adults and 280 times the maximum oral dose for children on a mg/ m2 basis).

Treatment

In case of overdose, it is recommended that the usual symptomatic and support measures be taken to remove the non-absorbed drug from the body.
Hemodialysis does not effectively remove fexofenadine hydrochloride from the blood.

SHELF LIFE

As indicated on the outer package

STORAGE

Store below 30ºC. Shake before use.
Discard any remaining contents 60 days after first opening the bottle.

MANUFACTURER

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Opella.